

Axcel®

PREDNISOLONE-5MG EC TABLET

COMPOSITION:

Each enteric-coated tablet contains
Prednisolone 5mg

PRESENTATION:

White to off white, round biconvex, enteric-coated tablet, embossed "5" on one side and plain on other side.

INDICATION:

Rheumatic fever, rheumatoid arthritis, allergic diseases, nephritic syndrome, bronchial asthma and other corticosteroid-indicated conditions.

PHARMACOLOGY:

Mechanism of Action:

Anti-inflammatory (steroidal)-Glucocorticoids decrease or prevent tissue responses to inflammatory processes, thereby reducing development of symptoms of inflammation without affecting the underlying cause. Glucocorticoids inhibit accumulation of inflammatory cells, including macrophages and leukocytes, at sites of inflammation. They also inhibit phagocytosis, lysosomal enzyme release and synthesis release of several chemical mediators of inflammation. Although the exact mechanisms are not completely understood, actions that may contribute significantly to these effects include blockade of the action of macrophage inhibitory factor (MIF), leading to inhibition of macrophage localization; reduction of dilation and permeability of inflamed capillaries and reduction of leukocyte adherence to the capillary endothelium, leading to inhibition of both leukocyte migration and edema formation; and increased synthesis of lipomodulin (macrocortin), an inhibitor of phospholipase A2 arachidonic acid release from the membrane phospholipids, with subsequent inhibition of the synthesis of arachidonic acid-derived mediators of inflammation (prostaglandins, thromboxanes and leukotrienes) immunosuppressant action may also contribute significantly to the inflammatory effect, involving prevention or suppression of cell-mediated (delayed hypersensitivity) immune reactions as well as more specific actions affecting the human response.

Pharmacokinetics:

Prednisolone is readily absorbed from the

gastrointestinal tract and already exist in a metabolically active form. Peak plasma concentrations of prednisolone are obtained 1 or 2 hours after administration by mouth, and it has an usual plasma half-life of 2 to 4 hours. Its initial absorption, but not its overall bioavailability, is affected by food. Prednisolone is extensively bound to plasma protein although less so than hydrocortisone (cortisol). Prednisolone is excreted in the urine as free and conjugated metabolites, together with an appreciable proportion of unchanged prednisolone crosses the placenta and small amounts are excreted in breast milk.

DOSAGE AND ADMINISTRATION:

Should be taken with food (take immediately after meal).

Usual adult and adolescent dose:

Oral, 5 - 60mg daily in divided doses.
For acute exacerbations of multiple sclerosis - oral, 200mg per day for one week, followed by 80mg every other day for one month.

Usual adult prescribing limits:

Oral, up to 250mg a day.

Usual pediatric dose:

Adrenocortical insufficiency - oral, 140mcg (0.14mg) per kg of body weight or 4mg per square meter of body surface a day in three divided doses.
Other indications - oral, 500mcg (0.5mg) to 2mg per kg of body weight or 15 to 60mg per square meter of body surface a day in three or four divided doses.

ROUTE OF ADMINISTRATION:

For oral administration only.

CONTRAINDICATIONS:

Osteoporosis, active peptic ulcer, psychoses or severe psychoneurosis. Systemic fungal infections. Acute infections unless effective, specific concurrent chemotherapy can be administered. Patients should not undergo immunizations. Tuberculosis as adjunctive treatment with tuberculostatic drug. Pregnancy and lactation. Should be used with great caution in the presence of congestive heart failure or hypertension, in patients with diabetes mellitus, epilepsy, glaucoma, infection diseases, ocular herpes simplex, chronic renal failure, and uraemia and in elderly persons.

PRECAUTIONS:

Extreme caution in congestive heart failure, diabetes mellitus, chronic renal failure, hypertension, recent intestinal anastomoses, infectious diseases, uraemia, elderly persons, diverticulitis, myasthenia gravis and ocular herpes simplex (corneal perforation may occur). History of peptic ulcer or psychotic tendencies; family history of diabetes mellitus; patient with previous tubercular infection should receive Chemoprophylaxis if corticosteroid therapy is prolonged. Monitor spinal radiographs for signs of osteoporosis; rheumatoid arthritis patients and postmenopausal women are at special risk. Growth inhibition or arrest, which cannot be overcome with exogenous growth hormone, may occur in children given

relatively low doses. Prolonged therapy may produce posterior sub-capsular cataracts. Glaucoma with possible damage to optic nerve. Steroid myopathy and Cushing's habitus may develop with long term therapy. Hepatic cirrhosis and hypothyroidism may enhance corticosteroid effects. Withdrawal syndrome (abdominal or back pain, dizziness, fainting, low grade fever, prolonged loss of appetite, muscle or joint pain, nausea, vomiting, shortness of breath, frequent or continuing unexplained headaches, unusual tiredness or weakness, rapid weight loss, reappearance of disease symptoms). Caution is required in patients with systemic sclerosis because of an increased incidence of (possibly fatal) scleroderma renal crisis with hypertension and decreased urinary output observed with a daily dose of 15mg or more prednisolone.

Pheochromocytoma crisis, which may be fatal, has been reported after administration of systemic corticosteroids. Corticosteroids should only be administered to patients with suspected or identified pheochromocytoma after an appropriate risk/benefit evaluation.

Use in Pregnancy and Lactation:

Pregnancy / Reproduction:

Fertility: Corticosteroids have been reported to increase or decrease the number or motility of spermatozoa. However, it is not known whether reproductive capacity in humans is adversely affected.

Pregnancy: Corticosteroids cross the placenta. Although adequate studies have not been done in humans, there is some evidence that pharmacologic doses of corticosteroids may increase the risk of placental insufficiency, decreased birthweight, or stillbirth. However, teratogenic effects in humans have not been confirmed. Infants born to mothers who received substantial doses of corticosteroid during pregnancy should be carefully observed for signs of hypoadrenalism and replacement therapy administered as required. Studies in animals have shown that corticosteroids increase the incidence of cleft palate, placental insufficiency, spontaneous abortion and intrauterine growth retardation. (FDA Pregnancy Category C).

SIDE EFFECTS:

The risk of adverse effects with pharmacologic doses of corticosteroid generally increases with the duration of therapy and frequency of administration and to a lesser extent, with dosage. The following side effects may occur: Cushing's syndrome, acne, hirsutism, osteoporosis, aseptic osteonecrosis, growth disturbances in children, myopathy, susceptibility of infections, gastric and intestinal ulcers with bleeding, hyperglycemia, glaucoma, posterior subcapsular cataract, physis disturbances, increased appetite, hypokalemia, unusual bruising wounds that will not heal, striae, cutaneous or subcutaneous tissue atrophy, hypopigmentation, dizziness or lightheadedness, flushing of face and cheeks, increased sweating, unusual increase in hair growth on body or face.

DRUG INTERACTION:

Response to corticosteroids is reduced by concurrent administration of barbiturates, phenylbutazone, phenytoin

or rifampicin. Response to anticoagulants and salicylates is reduced by corticosteroid therapy produces acute adrenal insufficiency which may be life-threatening. Co-treatment with CYP3A inhibitors, including cobicistat-containing products, is expected to increase the risk of systemic side-effects. The combination should be avoided unless the benefit outweighs the increased risk of systemic corticosteroid side-effects, in which case patients should be monitored for systemic corticosteroid side-effects. Some medicines may increase the effects of Axcel Prednisolone-5mg EC Tablet and your doctor may wish to monitor you carefully if you are taking these medicines (including some medicines for HIV such as cobicistat).

OVERDOSAGE AND TREATMENT:

For acute oral overdose-since there is no specific antidote available and serious adverse effects are unlikely, treatment consists of dilution with fluids.

STORAGE:

Store below 30°C. Protect from light.

KEEP OUT OF REACH OF CHILDREN JAUHI DARI KANAK-KANAK

PACK QUANTITIES:

Available in blister pack of 10 x 10's & 10 x 30's.

Further information can be obtained from pharmacist, physician or the manufacturer.

Manufactured By & Product Registration Holder :



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MAL 12020029AZ

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